جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : نجلاء حسن اسماعيل علي

المحافظة : الجيزه **الحي / المدينة :** البدرشين **العنوان :** قريه ميت رهينه

رقم البطاقة : 28111262104062 **رقم التليفون :** 01015144836 **تاريخ تسجيل الإستمارة :** 18-10-2026

العمر(تاريخ الميلاد) : 26-11-1981 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**

التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام رنين مغناطيسي علي المخ والحجاج بالصبغه [11112] **قسم العلاج :** الاورام

التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: نجلاء حسن اسماعيل علي
National ID: 28111262104062

GUSTAVE ROUSSY INTERNATIONAL EGYPT

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced MRI of the brain and orbits

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]



إعاده رفع

رنين مخي المخ بالصبغه

MRI Brain



National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Name: نجلاء حسن اسماعيل علي	
MR. : 4583	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالتها

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انها

metastaic HER2 +ve breast cancer to brain, ended WCI. for C1 Tykerb - Xeloda

44 YEARS OLD FEMALE PT WITH RT BREAST CANCER IDC GII ER 7/8 PR 5/8 HER2+3 KI6725% PT RECVIED NACT 4EC+4HP RT MRM PORT 10/2024 ADJUVANT PHESGO LAST 25/6/2025 TAM TILL 9/2025 BRAIN METS MRI BRAIN 17/9/2025 LT CEREBELLAR +LT TP MOSTLY METSTATIC SOLS 3.5 CM AT AT LT CEREBELLAR HEMISPHERE SURROUNDED BY EDEMA ANOTHER MASS AT TEMPOROPARIETAL REGION 4*2 CM MDT AT 27/9/2025 BRAIN PALLIATICE RT CT CAP +BONE SCAN START XELODA +TYKERB PT RECEVIED WCI 10 FX AT 10/2025 ONE CYCLE OF XELODA+TYKERB BEFOR ASSESMENT CT CAP 8/3/2026 COMPARED TO(26/10/2025) NEWLY DVELOPED HFL FOR TRIPHASIC OR DYNAMIC MRI HFL 1.6/1.2 CM SD OF REST OF STUDY STILL NOTED RT UPPER LOBAR TINY NODULE MEASURES ABOUT 2 MM EXAMINED SKELETON STILL NOTED D5+D12+L3 STRAITED LESIONSS LIKELY HEMANGIOMAS FOR MRI MRI BRAIN 8/3/2026 LT CEREBRAL METASTATI DEPOSITS MEASURING 2*1.2 CM ANOTHER AT LT PARIETAL REGION MEASUIRING 3*1.6 CM FOR COMPARISON PT RECEVIED ONLY ONE CYCLES XELOODA+TYKERB BEFOR ASSSMENT C3 XELODA+TYKERB AT 26/3/2026 TOTAL BILIRUBIN 1.43 ALT 14 AST 20 FOR MDT C3 XELODA3+3*14 TYKERB 1*5*30 LIVER SUPPORT

for C6 XELODA TYKERB (labs accepted) last MDT > for APUS + LFT + reconsult in clinic (not done but pt already received 6 cycles, for new CT CAP + MRI BRAIN WITH COMPARISON)

CT CAP + MRI BRAIN >> PENDING XELODA 3*3*14 TYKERB 5 TABS LABS OK

MRI BRAIN + CT CAP 6\2026 >> DONE BUT WITHOUT COMPARISON >> REQUESTED

CTS 6-2026: stationary xeloda 3x2x14 tykerb 5x1x30

Pt has Rt arm cellulitis >> refereed to ER today for xeloda 3x2x14 labs >> OK Tykerb 5-1-30 (16-9-2026) > Ph

تحريرا في ,

التاريخ :	19-09-2026	**الوقت :**	

الطبيب المعالج : اسماء محمد مصطفي نصر

يعتمد ,

جوستافروسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنت المصرية هي صحة مصر

MDT FORM 1 / 1 /2025 G

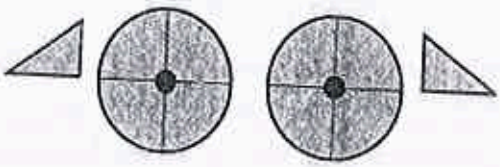
Name	نجلاء حسن اسماعيل	MRN	193/2024
Age	46 yrs	ID	٢٩١١٠٦٢١٠٤٧٤
Menopausal status	pre	Phone	٠١٠١٥٤٨٢٦
NRS SCORE		Marital status	M
BMI		Family history	-ve
CO morbidity	DM	REASON OF MDT	Mets to brain
		PS II	

Clinical Data

BX: IDC, GII ($T_4N_2M_0$)

ER:	PR:	HER 2:	Ki67:
7/8	5/8	+3	25%

R L



Sonomammography:

Mets Workup:

(2024) Pt ReCieved NACth EC X4 + TaX.L X4 TNM Brain + 4 phesgo

Then underwent Rt MRM → Then ReCieved poRT (10-2024) (17 Cycles) + TAM → (Still on TAM!!) and adjuvant phesgo

Recommendations:

(9/2025) MRI brain :- lt cerebellar + lt temporal SOL, (Met-stic)

BS (22/10/2025) :- Free

CT CAP (26/10/2025) :- No hepatic, pulmonary or osseous mets

MDT OUTCOME & PLAN

- Pt go for brain palliative RTh (ongoing)
- For MDT

For: Tykerb + Xelada + Zoladex

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT Form
[[DOCTYPE:MDT_FORM]]

GUSTAVE ROUSSY
INTERNATIONAL
EGYPTE

Name : naglaa haasan ismael
Branch : Portable
Ref.Physician Prof : ct cap+c

Date : 08-03-2026 11:01
Medical No. : GRI-46333
Ins.Company :

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

A known case of metastatic breast cancer, compared to the last CT report 26/10/2025, the current examination revealed

I- CHEST:

- Right MRM showed clear operative bed with no enhanced focal lesions or fluid collection.
- Still noted right upper lobar tiny nodule measures about 2mm.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation.
- Newly developed left hepatic lobe segment III/IVb non enhanced focal lesion measures about 1.6x1.2 cm for further triphasic CT /dynamic MRI
- No dilated biliary radicles.
- The gall bladder is well distended, average wall thickness with no stones.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No left stones or back pressure changes.
- Newly developed right upper and middle calyceal stones, the largest is seen in upper calyx measures about 1.28 cm
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- The examined skeleton still noted D5, D12 and L3 striated lesions likely hemangiomas for MRI correlation

Opinion:

- **Newly developed left hepatic focal lesion for further triphasic CT/Dynamic MRI.**
- **Stationary course regarding the rest of the examination.**

Dr.Norhan Ramzy,MD

Dr.NOURHAN RAMZY

Dr.NOURHAN RAMZY

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient Name	: الأستاذه / نجلاء حسن اسماعيل علي	Request No.	: 13006401
Patient No.	: 28111262104062	Request Date	: 03/01/2024
Path No.	: 25/24	Verification Date	: 21/01/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 42 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data:
Right axillary lymph node, Right breast areolar skin lesion.

Nature of Specimen:
Tru-cut biopsies, collected at 02/01/2024, 1:44 pm

Gross Examination:
1- Right axillary lymph node:
Three greyish white tissue cores up to 1 cm , totally submitted.
2- Right breast areolar and skin lesion:
Multiple greyish white tissue cores up to 1.8 cm , totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system
for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:
1- Right axillary lymph node:
* Histological type: Metastatic adenocarcinoma of primary mammry origin.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
* IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in <65% of tumor cells (Allred score 7/8).
- PR: Strong nuclear reaction in 25% of tumor cells (Allred score 6/8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
- Ki67: 22% nuclear reaction of tumor cells.
2- Right breast areolar and skin lesion:
* Histological type: Invasive duct carcinoma with ulceration.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
* DCIS: Minimal component.

Verified by :
dr.esraa magdy

CAP ACCREDITED
غرفة رقم (4) – الدور الاول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	:الأستاذه / نجلاء حسن اسماعيل علي	Request No.	:13006401
Patient No.	:28111262104062	Request Date	:03/01/2024
Path No.	:25/24	Verification Date	:21/01/2024
Sex	:Female	Financial Category	:اورام دار السلام - نفقة الدولة
Age	:42 Y	Department	:Out Patient
Referreing Doc	:	Registered by	:.marwa mohamed youss

Pathology Report

*Angioinvasion: Could not be detected within the limits of biopsy recieved
* IPT (Positive control showed appropriate reactivity):
- ER: Moderate nuclear reaction in >65% of tumor cells (Allred score 7/8).
- PR: Mild nuclear reaction in 60% of tumor cells (Allred score 5/8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
- Ki67: 25% nuclear reaction of tumor cells.

Diagnosis:
1- Right axillary lymph node, Tru-cut biopsy:
Metastatic adenocarcinoma of primary mammary origin.
IPT:
- ER: Positive (score 7/8).
- PR: Positive (score 6/8).
- Her2: Positive (score 3+).
- Ki67: Positive in 22% of tumor cells.
2- Right breast areolar and skin lesion, Tru-cut biopsy:
Invasive duct carcinoma, Grade II with ulceration.
IPT:
- ER: Positive (score 7/8).
- PR: Positive (score 5/8).
- Her2: Positive (score 3+).
- Ki67: Positive in 25% of tumor cells.

Signature
Prof. Dr. Lubna Omar
Dr. Heba Hasan

Verified by :
dr.esraa magdy

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / نجلاء حسن اسماعيل علي		
Patient No.	: 28111262104062	Request No.	: 13009821
Path No.	: 3426/24	Request Date	: 12/09/2024
Sex	: Female	Verification Date	: 23/09/2024
Age	: 42 Y	Financial Category	: اورام دار السلام - نفقة الدولة
Referreing Doc	:	Department	: Out Patient
		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast cancer, post NACT.
Nature Specimen: Right modified radical mastectomy specimen.

Gross Examination:
* Procedure: Right MRM measured 29x17x6 cm with covering skin ellipse measured 29x17 cm.
* Tumor extent: An axillary tail greyish white tumor bed site measured 3x2x1.5 cm.
* Another greyish white area 8x5x4 cm, 4 cm away from the axillary tail mass.
* Skin involvement: Nipple was flat grossly.
* Distance from margins: 1.5 cm away from deep margin.
* Lymph nodes number and size: 9 L.Ns up to 1.5x1x0.5 cm.
* Also received in the same container separate fibrofatty tissue piece measured 8x6x1 cm looked unremarkable grossly.

* Designation:
1-3: Axillary tail T.B.S
4: Area in between
5-12: The G.W area.
13: The nipple and areola.
14: The least deep margin.
15: The separate tissue.
16-18: Axillary lymph nodes.
Grossing by: Dr. Duaa Mohamed.

Microscopic Examination:
* Histological type: No residual invasive tumor tissue.
* Ductal carcinoma in situ: Negative.
* Skin involvement: Free nipple and areola with eczematous changes and Toker cells

Verified by :
Dr.Toka Medhat

CAP
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	:الأستاذه / نجلاء حسن اسماعيل علي	Request No.	:13009821
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Sex	:Female	Financial Category	:اورام دار السلام - نفقة الدولة
Age	:42 Y	Department	:Out Patient
Referreing Doc :		Registered by	:Mrs.Mayada Hegazy

Pathology Report

hyperplasia.
* Lymph node involvement: Negative (0/9) with therapy effect.
* Resection Margins: Free deep margin.
* Lymphovascular invasion: Negative.
* Perineural invasion: Negative.
* Histologic response to therapy: complete pathological response.
* Microcalcifications: Negative.

Diagnosis:
Right breast cancer post NACT, MRM specimen:
* No viable tumor in a previously diagnosed invasive duct carcinoma.
* No DCIS.
* Free deep margin.
* Free ALNs (0/9).
* Free nipple and areola skin.
*Pathologic complete response.
- Stage: ypT0, ypN0.

Signature
Dr. Walaa Ghonaim.
Dr. Toka Medhat.

Verified by :
Dr.Toka Medhat

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Naglaa Hassan Ismail Ali	Date : 18/04/2024
Exam : C.T CHEST	Age : 42 Year(s)
Medical ID: 193/2024	Pat. ID: 46333

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST: -

Findings:

- Clear both lung fields
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- No gross cardiac abnormality. No pericardial effusion detected.
- Normal non-enhanced appearance of the thoracic aorta and the great vessels.
- Upper abdominal cuts are unremarkable.

OPINION:

- ***Essentially normal CT study of the chest.***

Dr. Mariam Sabri

Dr. Amr Farouk, M.D.

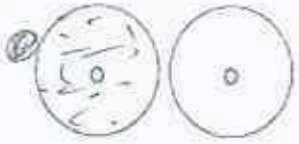
CT Scan
[[DOCTYPE:CT_SCAN]]

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : نجلاء حسن اسماعيل على
Sex: Female
3- Medical number: 83183385
Old Med : 193/2023
2-ID number: 26111262104062
Patient Age : 42 Y



Family history		Phone	01033490672	BMI		REASON OF MDT	
marital status		NPS Score		GROUP	5	Date of MDT	01/02/2024
Co morbidity	medically free	Menopausal status		Initial STAGE			

Clinical data

Breast Imaging :
BSM : RT Breast distention at RT accessory breast with ulcerated nipple
Two pathologically proven level 1 axillary LN lowgat 2.2*1.5 cm

Mets Workup :

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC G2 with ulceration	7/8	5/8	3		25%

T	N	M
4D	2	0

Sonomammography

PATIENT HISTORY
ECHO
LABS
BONE SCAN

MDT OUTCOME & PLAN

NACT 4 EC, 4 THP WITH ZOLADEX (OV FUNCTION PRESERVATION) THEN RIGHT MRM

SIGNATURE

MEDICAL ONCOLOGY		RADIOLOGY	
SURGERY		PATHOLOGY	
Psychiatric		Nutrition	

Patient Name: Naglaa Hasan Ismael	Patient ID: 55350
Age: 45 Years	Study Date: 24/1/2024
Ref. Phy. Dr.: Dear Doctor	Sex: F

Multislice non contrast CT scan of the chest, abdomen and pelvis with multiplannar reformatted images revealed:

I- Chest:

- **Right breast mild skin thickening with right axillary pathological LNs the larger 1.9 cm for US correlation.**
- Both lungs are clear. No focal pulmonary parenchymatous lesions seen.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.
- ***Small sliding hiatus hernia.***

II- Abdomen and pelvis:

- Average hepatic size showing homogeneous parenchymal pattern. ***No hepatic lesions*** or biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, adrenals, both kidneys, aorta, IVC & urinary bladder.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- ***No suspicious bony lesions by CT study***

Best regards,
Dr. Ahmed Gabr , MD

CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
لجنة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Naglaa Hassan Ismail Ali	Date : 15/08/2024
Exam : MAMMOGRAPHY BOTH	Age : 42 Year(s)
Medical ID: 193/2024	Pat. ID: 46333

Combined bilateral breast mammography and complementary ultrasound

Indication:

Right breast pathologically proven malignant mass on NACT for assessment.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Predominant fatty density (ACR a)
- Right breast diffuse increase skin thickness.
- No spiculated masses or suspicious microcalcifcations.
- Complementary ultrasound revealed no solid masses or cysts.
- The right nipple is retracted with no ulceration at today's study.
- Normal skin thickness, and nipple-areola complex of the right breast.
- Nonspecific axillary LN, bilaterally
- Free left infra and supra-clavicular regions.

Opinion:

- Complete radiological response of the right malignant lesions (BIRADS 6)
- Benign findings right breast (BIRADS 2)

Yours truly

Signature Dr. Asmaa Zenhom

15/08/2024 4:29 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Naglaa Hassan Ismail Ali	Date : 18/04/2024
Exam : C.T CHEST .	Age : 42 Year(s)
Medical ID: 193/2024	Pat. ID: 46333

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Dr. Mariam Sabri

Dr. Amr Farouk, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]

Cairo University
National Cancer Institute
Department of Diagnostic and
Interventional Radiology

جامعة القاهرة
المعهد القومي للأورام
قسم الأشعة التشخيصية والتداخلية

Exam Information:

Patient Name: NAGLA Hassan	*Date of birth: 26/11/1981*
Arabic Name: نجلاء حسن	*Patient Gender: F*
Patient RIS ID: 351191	*Exam Date: 17/03/2024*
Patient HIS ID: 124005290	
Written by:	Approved by:
Dr. Hoda Fathy	Dr. Hoda Fathy

^{99m}Tc MDP Bone Scan Report (17/03/2024)

** Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of ^{99m}Tc MDP

Revealed:

- Multiple scattered well defined warm foci are seen over DV3, posterior portions of left 6th and bilateral 9th ribs?? Areas of insufficiency fractures (looser zones) for CT correlation to confirm/exclude their nature.
- Otherwise, No other significant active osseous lesions could be detected all over the rest of the surveyed skeleton.
- Arthritic changes are noted over both shoulder and both knee joints.
- Degenerative changes are noted over the dorso-lumbar vertebrae with LV5 facet arthropathy
- Both kidneys are visualized.

Conclusion:

Multiple equivocal scattered osseous lesions as described for CT cuts correlation.

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Patient Name: Naglaa Hasan Ismael	Patient ID: 55350
Age: 45 Years	Study Date: 24/1/2024
Ref. Phy. Dr.: Dear Doctor	Sex: F

Multislice non contrast CT scan of the chest, abdomen and pelvis with multiplannar reformatted images revealed:

I- Chest:

- **Right breast mild skin thickening with right axillary pathological LNs the larger 1.9 cm for US correlation.**
- Both lungs are clear. No focal pulmonary parenchymatous lesions seen.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.
- ***Small sliding hiatus hernia.***

II- Abdomen and pelvis:

- Average hepatic size showing homogenous parenchymal pattern. ***No hepatic lesions*** or biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, adrenals, both kidneys, aorta, IVC & urinary bladder.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- ***No suspicious bony lesions by CT study***

Best regards,
Dr. Ahmed Gabr , MD

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

Patient Name: Naglaa Hassan Ismail Ali	Date : 26/10/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 44 Year(s)
Medical ID: 193/2024	Pat. ID: 46333

Technique: CT scan of the chest , abdomen and pelvis obtained post IV contrast administration. Oral contrast not given.

Limitation: Motion artifact .

Findings:

**** Chest :-**

* A small calcified nodule is seen in upper lobe of right lung , likely post granulomatous .
* No masses or cavitating pulmonary lesions.
* Clear both costophrenic recesses
* No enlarged hilar or other mediastinal lymphadenopathy or other mediastinal masses .
* Average cardiac size . No pericardial sac collection .

****ABDOMEN & PELVIS :-**

*Mild enlarged liver showing homogenous density . No C.T. evidence of hepatic focal lesions and No dilated intra or extra-hepatic biliary ducts .
*Patent main trunk and both branchs of the portal vein .
*Normal C.T. appearance of the spleen , gall bladder , pancreas , adrenal glands , pelvic organs , aorta and I.V.C.
*Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
* IUD is seen in place .
*No retroperitoneal lymphadenopathy .
*No ascites .

****EXAMINED SKELETON :-**

* D5 , D12 and L3 vertebrae show vertical linear pattern , corduroy sign characteristic of vertebral hemangioma .
*No lytic or sclerotic bony lesions in the scanned bones .

****OPINION :**

*** A case of right MRM showing ;**
- Right upper lobe small calcified lung nodule .
- Mild hepatomegaly , no focal lesions .
- D5 , D12 and L3 vertebral heamangioma .
- Unremarkable rest of the study .

Signature Reeham.Khader
29/10/2025 2:25 AM

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

Name : naglaa haasan ismael	**Date : 08-03-2026 11:01**
Branch : Portable	**Medical No. : GRI-46333**
Ref.Physician Prof : ct cap+c	**Ins.Company :**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

A known case of metastatic breast cancer, compared to the last CT report 26/10/2025, the current examination revealed

I- CHEST:

- Right MRM showed clear operative bed with no enhanced focal lesions or fluid collection.
- Still noted right upper lobar tiny nodule measures about 2mm.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation.
- Newly developed left hepatic lobe segment III/IVb non enhanced focal lesion measures about 1.6x1.2 cm **for further triphasic CT /dynamic MRI**
- No dilated biliary radicles.
- The gall bladder is well distended, average wall thickness with no stones.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No left stones or back pressure changes.
- Newly developed right upper and middle calyceal stones, the largest is seen in upper calyx measures about 1.28 cm
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- **The examined skeleton** still noted D5, D12 and L3 striated lesions likely hemangiomas for MRI correlation

Opinion:

- **Newly developed left hepatic focal lesion for further triphasic CT/Dynamic MRI.**
- **Stationary course regarding the rest of the examination.**

Dr.Norhan Ramzy,MD

Dr.NOURHAN RAMZY

Dr.NOURHAN RAMZY

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

Name : NAGLAA HASAN ESMAEEL	**Date : 08-03-2026 10:24**
Branch : Portable	**Medical No. : GRI-46333**
Ref.Physician Prof : MRI BRAN + C	**Ins.Company :**

OPEN MR EXAMINATION OF THE BRAIN

TECHNIQUE:



Axial T1, T2, FLAIR, diffusion & ADC WIs
Coronal T2WIs
Sagittal T1WIs
Post contrast WIs

FINDINGS:

- A metastatic brain deposit is seen in the left cerebellar region measuring about 2 x 1.2 cm as well as another larger left parietal region lesion measuring about 3 x 1.6 cm in its maximal diameters. The lesions elicit low signal in T1, high signal in T2 and FLAIR WIs with marginal post contrast enhancement. Minimal perifocal edema around the lesions with minimal mass effect on the adjacent structures.
- No mid line shift.
- Symmetrical appearance of the ventricular system.
- No intra or extra axial areas of fresh blood signal intensities or its degradation products.
- The brain stem is normal.

OPINION:

Left cerebellar and left cerebral metastatic deposits as described, for follow up kindly correlate with unavailable previous studies.

Much Obliged;
Dr.Marwa Elgawish, MSc, FEBR

M.R.I
[[DOCTYPE:MRI]]